National Cancer Control Programme (NCCP) 00892 · clinician draft

Adjuvant ribociclib + endocrine therapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Adjuvant ribociclib + endocrine therapy · supportive-care plan

Active medicines: Ribociclib + Aromatase inhibitor ± ovarian suppression

Emetogenic risk: Oral moderate–high

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP classifies ribociclib as oral moderate–high emetic risk. Agree preventive or rescue treatment under local policy after QT and CYP3A interaction review; no default QT-prolonging antiemetic is selected by this information card.
- Attend the baseline and cycle 1 Day-14 electrocardiogram (ECG) and scheduled electrolyte checks. Fainting, palpitations or severe dizziness need urgent assessment.
- Keep two-weekly blood tests during cycles 1–2 and pre-cycle tests during cycles 3–6. Report fever, jaundice, dark urine or new breathing/skin symptoms promptly.
- The aromatase inhibitor continues during the ribociclib rest week and generally after ribociclib completion. Discuss menopause, joints, sexual health, bone risk and any injection-site symptoms from ovarian suppression.
- Ribociclib has a planned maximum three-year course. Confirm the endocrine duration and early-breast surveillance plan separately; new symptoms should be assessed without waiting for a routine appointment.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00892 v2 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required